Sample grant concept note

FLUID-BAL: fluid balance and survival in ICU sepsis

Foundations of Clinical Research · 2-page concept note with funder tips

FICTIONAL EXAMPLE FOR TRAINING

Invented for teaching: the study, team and budget are not real. The right-hand column shows what funders look for in each section.

Funder / call: [Funder name], small research grants for health professionals, [year]

Applicant: [Name], consultant intensivist, Department of Intensive Care, [Institution]

Amount / duration: USD 18,040 · 24 months

1 Problem

Sepsis is one of the commonest reasons for ICU admission at [Institution], with about 250 adult cases a year and a 28-day mortality of about one in four. Patients receive large volumes of intravenous fluid, and studies elsewhere link a positive fluid balance to death. But these studies cannot separate the effect of fluid from the effect of being sicker, and we have no data on our own practice. Without them we cannot design a trial of fluid restriction or improve fluid prescribing.

Funder tip

State the size of the problem with numbers, then the gap, in under 120 words. Local data beat general statements.

2 Objectives

Primary: to estimate the association between cumulative fluid balance at day 5 (ml/kg) and 28-day mortality, adjusted for severity. Secondary: to describe where fluid comes from (including hidden sources) and to measure the completeness of fluid charts.

Funder tip

One primary objective, one or two secondary ones. Each must be measurable and match the budget.

3 Approach

Prospective cohort of about 320 consecutive adults with sepsis over 18 months. Trained ICU nurses record daily fluid input and output and severity scores from routine charts on a piloted CRF. Logistic regression adjusted for age, SOFA, lactate and infection source. No change to care; ethics approval with a consent waiver is being sought.

Funder tip

Show that the design fits the question and that it is feasible: patients per month, who collects data, how bias is handled.

4 Expected outcomes and impact

A peer-reviewed paper (STROBE); a fluid-practice report and teaching session for ICU staff; a revised fluid chart; and the effect size, recruitment rate and data completeness needed to plan a multicentre trial of fluid restriction.

Funder tip

List concrete outputs and who will use them. Funders like a clear "next step".

5 Team

Consultant intensivist (PI, 10% time); ICU nurse manager (data collection lead); clinical pharmacist (fluid prescriptions); biostatistician from the [Institution] research office. Two team members completed a clinical research methods course in [year].

Funder tip

Show the skills match the tasks. Naming a statistician from the start is a strong signal.

6 Timeline

Months 1–3: ethics approval, registry entry, CRF pilot and nurse training

Months 4–21: recruitment and day-28 follow-up; quarterly data-quality reports

Months 22–24: analysis, staff feedback, manuscript submission

Funder tip

Be realistic: approvals and recruitment usually take longer than planned. Show milestones.

7 Budget summary (USD)

Research nurse time (0.2 FTE × 18 months) 9,000 · Training and printing 600 · Data management 1,500 · Statistical support 2,500 · Ethics fee 300 · Open-access fee 2,500 · Contingency (10%) 1,640
Total: USD 18,040 · In kind from [Institution]: PI time, office space, IT.

Funder tip
Every line should link to an activity. Show what your institution contributes. Avoid "miscellaneous".

8 Why fund this now?

The study is low-cost, needs no new equipment and uses data already collected. It turns routine charts into evidence and produces exactly what is needed to apply for a larger trial grant.

Funder tip
End with value for money and what the grant will lead to next.